

From People as Recipients to Partners in Health Creation: A Perspective on Bridging the Evidence-to-Health Gap

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1 Abstract

Despite unprecedented advances in biomedical science, clinical research and evidence-based healthcare, improvements in population health have frequently fallen short of expectations. This persistent disconnect between the generation of evidence and the realization of better health outcomes may be understood as an **Evidence-to-Health Gap** the gap between what is known to improve health and what people are ultimately able and willing to implement in their daily lives. While considerable effort has focused on strengthening evidence generation, dissemination and implementation, comparatively less attention has been given to the central role of people as the primary agents through whom health is ultimately created and preserved in their daily lives. People remain central to behaviour change, which is influenced by capability, opportunity and motivation, as conceptualised in the COM-B model. Yet, people are often viewed as consumers of evidence rather than partners in creating evidence-informed health. This Perspective proposes partnership with people as a unifying principle that connects evidence generation, knowledge translation, implementation and health creation.

We hypothesise that the persistent Evidence-to-Health Gap may be partly explained by insufficient partnership with people throughout the evidence-to-action continuum. We thus propose that instead of viewing individuals primarily as recipients of healthcare, or as participants in episodic clinical decision making or the targets of behavioural interventions, people's central role in health should be acknowledged by actively involving them throughout the evidence-to-action continuum. This we propose through a broader partnership framework in which people participate in not only in applying evidence but also in defining the problems, asking the questions, generating the evidence, translating the knowledge and continuously improving the actions through which healthier lives are created. Scientific research, healthcare professionals and health systems should aim to support, enable and strengthen people's capacity to create, preserve and restore health.

This perspective integrates concepts from evidence-based medicine, lifestyle medicine, implementation science, person-centred care, public health, citizen science, shared decision making and learning health systems into a unified conceptual framework for collaborative health creation centred on partnership with the people that empowers them. The proposed framework extends beyond disease prevention and healthcare delivery towards improving Quality of life, abbreviated here as LifeQuality, with good health as one of the key components of overall wellbeing. Since rights and responsibilities are inseparable, sustainable health creation depends upon both upon empowerment and accountability. We conclude by outlining implications for research, policy and practice, suggesting a research agenda for evaluating partnership with people as the foundational principle for generating and translating scientific evidence into measurable improvements in individual and population health.

Keywords

Evidence-to-Health Gap; Health Creation; Partnership; Evidence Translation; Lifestyle Medicine; Implementation Science; Person-Centred Care; Shared Decision Making; Public Health; Citizen Science; Learning Health Systems; LifeQuality.

2 Introduction:

The past century has witnessed extraordinary advances in biomedical discovery, clinical medicine and public health [1] [2] . Vaccination, sanitation, antibiotics, advances in surgery, imaging, genomics and evidence-based

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therapeutics have transformed healthcare [2] and substantially improved life expectancy in many parts of the world. Simultaneously, rigorous methods for generating and evaluating scientific evidence have become firmly embedded within healthcare through evidence-based medicine, systematic reviews and clinical practice guidelines. [3] Collectively, these developments have strengthened the scientific foundations of healthcare and improved the capacity to generate, evaluate and implement evidence.

Yet, Non-communicable diseases continue to account for the majority of premature mortality worldwide, while unhealthy diets, physical inactivity, tobacco use, harmful alcohol consumption, inadequate sleep, chronic stress and adverse social determinants continue to drive much of the global burden of disease. Although effective preventive and therapeutic interventions for many of these determinants are already well established, their sustained adoption remains limited across populations. Despite these advances, the global burden of chronic non-communicable diseases continues to increase. Cardiovascular disease, diabetes, obesity, chronic respiratory disease, mental illness and several cancers remain leading contributors to disability and premature mortality worldwide. Although life expectancy has improved in many countries, gains in healthy life expectancy have been comparatively smaller, leaving many populations living longer but with greater chronic morbidity. Considerable scientific progress has therefore not translated into comparable improvements in population health.

The concept of health creation has received growing attention within public health and salutogenic models, emphasising assets, capabilities and the social conditions that enable individuals and communities to flourish rather than focusing exclusively on disease prevention. These perspectives build upon earlier health promotion traditions that emphasised enabling people to increase control over and improve their health. Lifestyle Medicine similarly demonstrates that many leading causes of morbidity and premature mortality are strongly influenced by modifiable behaviours supported by robust scientific evidence. Nevertheless, translating this evidence into durable behavioural change remains one of the greatest challenges in contemporary healthcare. Behaviour change theories and ecological models consistently demonstrate that knowledge alone rarely produces sustained action; individuals must navigate competing priorities, social influences, economic constraints, environmental barriers and personal motivations that shape health-related decisions over the life course.

Though much of modern healthcare remains organised around episodes of illness, clinical encounters and institutional delivery of services, yet they represent only a fraction of the circumstances in which health is created or lost. The biological consequences of nutrition, physical activity, sleep, stress management, substance use, social relationships, environmental exposures and adherence to evidence-informed recommendations accrue largely outside healthcare settings through decisions and behaviours enacted repeatedly in everyday life. This broader understanding of health has been recognised for decades through health promotion, ecological models of health behaviour and population health frameworks that emphasise the contributions of lifestyle, environment and social context to health outcomes. Consequently, the ultimate effectiveness of scientific discovery depends not only upon the quality of evidence generated but also upon people's capacity, motivation and opportunity to translate that evidence into sustained action. This observation has become increasingly evident across chronic diseases, where behavioural and lifestyle factors contribute substantially to both disease burden and health improvement.

This disconnect extends beyond prevention. Across many conditions, evidence-based recommendations often fail to achieve their anticipated impact because they are inconsistently implemented, incompletely adopted or insufficiently sustained in the settings where people actually live their lives. Important gaps remain between what scientific evidence demonstrates to be effective and the health that people ultimately experience in their daily lives. [4] [5] [6] Despite major advances in evidence-based medicine and healthcare quality, delays in translating research into practice remain common, and considerable research waste persists when evidence fails to influence real-world health. [2] [7] [8] [9] [10] [11] This persistent disconnect has been recognised in different forms as evidence-to-practice gaps, knowledge translation gaps and implementation gaps, but comparatively less attention has been given to the final translation of evidence into healthier lives. [3] [12] [13] [14]

Despite this complexity, one fundamental reality often receives surprisingly little explicit attention. Throughout the entire continuum of health creation from preventing disease and maintaining wellbeing to responding to illness, recovering from treatment and living with chronic conditions people themselves remain the principal agents. They make daily decisions about food, physical activity, sleep, substance use, stress, relationships and healthcare seeking. They are the first responders to symptoms, the primary managers of long-term conditions, the most frequent caregivers within families and communities, and ultimately the individuals who live with the consequences of both healthy and unhealthy choices.

This Perspective refers to this persistent disconnect as the **Evidence-to-Health Gap**: the gap between available scientific evidence and the creation, preservation and restoration of health in people's everyday lives. These observations suggest that the Evidence-to-Health Gap cannot be understood solely as a problem of evidence production or implementation. It may also reflect an incomplete conceptualisation of the role of people within the

health ecosystem. People are often engaged after evidence has been generated, when recommendations are communicated and implementation is expected. By contrast, comparatively less emphasis has been placed on partnering with people throughout the entire evidence-to-action continuum from identifying the questions that matter most, through evidence generation and synthesis, to contextual adaptation, implementation, evaluation and continuous improvement. Existing movements, including patient and public involvement in research, citizen science and community-engaged participatory research [15] [16] [17] [18] [19] [20], have demonstrated that meaningful partnership can improve the relevance, acceptability and uptake of research while strengthening trust between science and society. However, these initiatives remain fragmented across disciplines and have not yet been integrated into a comprehensive conceptual framework for health creation.

We therefore propose that the persistent **Evidence-to-Health Gap** may be partly explained by insufficient partnership with people throughout the evidence-to-action continuum. This proposition does not diminish the essential roles of scientific research, healthcare professionals or health systems. Rather, it repositions these institutions according to their fundamental purpose: to support, enable and strengthen people’s capacity to create, preserve and restore health as the primary agents of health creation. This Perspective therefore proposes partnership with people as a unifying principle that connects evidence generation, knowledge translation, implementation and health creation.

Within this perspective, partnership extends beyond participation or consultation. People participate not only in applying evidence but also in defining the problems, asking the questions, generating the evidence, translating the knowledge and continuously improving the actions through which healthier lives are created. Evidence should be synthesised into forms that are openly accessible, understandable and relatable to people’s situations and needs. Such synthesis requires not merely simplification but contextualisation, ensuring that scientific knowledge remains rigorous while becoming meaningful and actionable within diverse personal, cultural and socioeconomic circumstances.

The remainder of this perspective develops this partnership hypothesis, presents a conceptual model for bridging this Gap, situates the proposal within existing scholarly traditions, and discusses implications for research, policy and practice. Rather than replacing established paradigms, the framework seeks to integrate and extend them around a unifying principle: that sustainable improvements in health and thereby in LifeQuality depend upon recognising people not primarily as recipients of healthcare but as partners in health creation.

3 Explaining the Evidence-to-Health Gap (continued)

Many existing models implicitly position people near the end of the evidence pathway, after priorities have been established, research questions defined, evidence generated and recommendations formulated. People are then expected to adopt behaviours, adhere to treatments or comply with guidelines that have largely been developed without their continuous involvement [21] [19]. Although patient and public involvement in research has expanded considerably in recent years, meaningful partnership remains uneven across disciplines and stages of the research-to-practice continuum [17] [18] [22] [19].

Consequently, opportunities are frequently missed to improve the relevance, acceptability and usability of scientific evidence. Research priorities may differ from community priorities [23]. Outcome measures valued by investigators [21] may not fully capture what matters most to people living with chronic conditions. Evidence summaries may remain inaccessible [24] [3] to non-specialist audiences. Recommended interventions may be difficult to implement [25] [26] within the realities of work, family responsibilities or limited resources. These disconnects do not diminish the quality of the science itself; rather, they reduce its ability to generate measurable improvements in health.

Although numerous determinants contribute to the Evidence-to-Health Gap, they share a common feature: each represents a weakening of the connection between scientific knowledge and people’s capacity to translate that knowledge into healthier lives [27] [25]. Consequently, the challenge is not solely one of producing better evidence or implementing it more efficiently within healthcare systems [5] [13]. It is equally a challenge of enabling people to understand, evaluate, adapt and sustain evidence-informed actions within the realities of everyday living.

Our perspective therefore proposes that the Evidence-to-Health Gap should be viewed as a systems challenge extending across the entire evidence-to-action continuum. Scientific discovery, evidence synthesis [13] [3], guideline development [1], healthcare delivery, public health programmes [28], digital technologies, education, community organisations and people themselves all contribute to this continuum. Weakness at any stage diminishes the likelihood that evidence will ultimately improve health.

3.1 Central Partnership Hypothesis

This Perspective advances the following central hypothesis:

The persistent Evidence-to-Health Gap may be partly explained by insufficient partnership with people throughout the evidence-to-action continuum.

Specifically, we propose that sustainable improvements in health can be accelerated when people are recognised not merely as recipients of healthcare or adopters of evidence, but as active partners throughout the processes by which knowledge is created, translated [13], implemented [5] and continuously improved [6]. People participate not only in applying evidence but also in defining the problems, asking the questions, generating the evidence, translating the knowledge and continuously improving the actions through which healthier lives are created.

More importantly, the proposed framework extends beyond disease prevention and healthcare utilisation towards the broader objective of improving **LifeQuality**, as understood in terms of overall well-being with health as one of the determinant, which also includes education, dignity, opportunity, participation, security and rights.

Importantly, recognising the central role of people does not imply transferring responsibility for health exclusively to individuals. Health behaviours are shaped by biological, psychological, social [23] [29], economic, commercial and environmental influences, many of which lie beyond individual control. Governments, healthcare organisations, educational institutions, employers and communities therefore retain essential responsibilities for creating conditions that enable healthier choices [28]. The present Perspective argues not for individualisation of responsibility but for a more balanced partnership in which institutional responsibilities and personal agency are mutually reinforcing.

Since rights and responsibilities are inseparable, this framework recognises that people’s partnership combines respect with accountability: empowering people while acknowledging their shared responsibility for the choices through which health, and thus and LifeQuality are progressively created. Respect acknowledges individual autonomy, lived experience and the legitimate role of people in decisions affecting their health. Accountability recognises that opportunities for improved health are realised through actions taken by individuals, families, communities, professionals and institutions working together. When people are involved and consulted, they get better informed, and strengthens their sense of agency. This strengthens their Motivation, and Capability, key elements of the COM-B model of behaviour change.

People’s trust in the health system and provider network is often one of the constraints in adoption of health advice. A participatory framework for health builds trust from the beginning, which helps in consistent translation of evidence,

This hypothesis extends rather than replaces existing paradigms. Evidence-based medicine established the importance of integrating the best available evidence with clinical expertise and individual values [4] [30]. Implementation science has advanced methods for improving adoption of evidence-based interventions [5] [26] [31]. Shared decision making [21] has strengthened collaboration between clinicians and patients. Learning health systems have demonstrated the value of continuous organisational learning [6]. Lifestyle medicine has highlighted the importance of sustained health behaviours, while public health has long recognised the influence of social and environmental determinants. Together, these disciplines provide essential foundations upon which a broader partnership model can be built.

The distinctive contribution of this Perspective is to integrate these complementary developments within a single organising principle centred on health creation centered on people. Scientific research, healthcare professionals and health systems as enabling systems exist to support, enable and strengthen people’s capacity to create, preserve and restore health. Accordingly, partnership begins before evidence is generated and continues long after it is implemented. Research priorities become more relevant when informed by people’s lived experiences. Evidence becomes more applicable when generated with meaningful public participation. Translation becomes more effective when evidence is communicated in forms that are usable beyond professional audiences. Implementation becomes more sustainable when people are equipped to adapt evidence within their own lives and communities. Continuous improvement becomes possible when lived experience informs subsequent cycles of research, policy and practice.

4 Conceptual Model: Partnership Across the Evidence-to-Action Continuum

Traditional models frequently portray translation as a largely linear process in which research generates evidence, evidence informs guidelines, healthcare professionals deliver interventions, and people receive care [4] [13] [5]. While valuable, this sequence implicitly positions people near the end of the process, primarily as recipients of evidence or healthcare services.

If the Evidence-to-Health Gap is to be bridged, we propose that people be made central to all evidence generation and consumption, in a system that involves partnership with people throughout the evidence-to-action continuum. Once evidence becomes openly accessible, understandable, relatable and useful to people’s daily lives, it becomes actionable and thus helps in its adoption. Viewed in this context, scientific research, healthcare professionals and health systems would support, enable and strengthen people’s capacity to create, preserve and restore health.

The resulting model is neither hierarchical nor transactional. Instead, it represents a collaborative ecosystem in which each stakeholder contributes distinct but complementary capabilities. Researchers contribute scientific rigour and discovery. Healthcare professionals contribute clinical expertise, ethical practice and compassionate care. Health systems create structures that facilitate equitable, high-quality services. Public health agencies establish supportive policies and environments [28]. Communities provide social support [23] and collective capacity. People integrate all of these resources within the realities of everyday life through the choices and actions by which health is ultimately created.

The partnership model proposed here, thus, reconceptualises this pathway as a continuous, bidirectional and learning-oriented process in which people participate throughout the entire evidence-to-action continuum. Partnership begins with identifying the health challenges that matter most to individuals and communities, continues through evidence generation and interpretation [13], extends to the synthesis and communication of knowledge [3], and culminates in implementation, evaluation and continuous improvement [5] [6]. At every stage, people contribute unique experiential knowledge that complements scientific and professional expertise.

Importantly, the model recognises that evidence generation and evidence application are not separate activities [6] but components of a continuous learning cycle. Experiences arising from implementation generate new questions, identify practical barriers [26], reveal contextual influences and expose evidence gaps [14] that inform future research. Partnership therefore transforms people from passive recipients of scientific knowledge into active contributors to an iterative system of health creation and continuous learning.

Evidence should be synthesised into forms that are openly accessible, understandable and relatable to people’s situations and needs. Scientific evidence has little potential to improve health if it remains inaccessible, incomprehensible or disconnected from the contexts in which people make everyday decisions [24] [3]. Advances in evidence synthesis, plain-language communication [13], digital technologies and open science provide opportunities to transform complex scientific knowledge into practical resources that support informed action while preserving scientific integrity.

This conceptualisation aligns with contemporary developments in learning health systems [6], implementation science [5] [26] and participatory research [32] while extending each beyond its traditional boundaries. Rather than treating partnership as a discrete intervention or engagement strategy, the present model proposes partnership as the organising principle that connects these complementary disciplines into a coherent framework for translating evidence into health.

If this hypothesis is valid, its implications extend beyond healthcare delivery. It suggests that evidence generation, knowledge synthesis, professional education, health communication, digital health, community engagement and public policy should all be designed with partnership as a foundational principle. Such an orientation has the potential to strengthen not only implementation but also the relevance, accessibility, trustworthiness and sustainability of health improvement efforts across the entire evidence-to-action continuum.

Within this broader framework, the success of scientific evidence should ultimately be judged not only by publication, citation, guideline inclusion or implementation within healthcare systems, but by its contribution to healthier lives and improved LifeQuality. This shift in emphasis encourages greater alignment between research priorities, healthcare delivery, public health action and the lived realities of the people they are intended to serve.

4.1 Partnership with People Across the Evidence-to-Action Continuum Model

The proposed model comprises seven interconnected stages.

1. Identifying priorities

Health creation begins by understanding the needs, aspirations and challenges experienced by individuals and communities. Partnership at this stage ensures that research priorities and health policies address questions that matter most to the people whose health they are intended to improve [23].

2. Generating evidence

Scientific investigation remains the cornerstone of reliable knowledge [4]. Researchers, clinicians, communities and people collaborate, where appropriate, in designing studies, selecting meaningful outcomes [21] and ensuring that research addresses real-world contexts without compromising scientific rigour.

3. Synthesising knowledge

Evidence should be synthesised into forms that are openly accessible, understandable and relatable to people's situations and needs. Systematic reviews [13] [3], clinical guidelines, decision aids and digital knowledge platforms all contribute to this process, but effective synthesis also requires contextual interpretation that enables informed action. We propose a citizens platform for this pupose,

4. Shared interpretation and decision making

Evidence gains practical meaning through dialogue among people, healthcare professionals and other stakeholders. Scientific evidence, professional expertise and individual values are integrated [21] to determine actions that are both evidence-informed and personally relevant.

5. Health creation in daily life

The central stage of the model occurs beyond formal healthcare settings, in peoples homes and daily lives where health is ultimately created through everyday decisions [11] [2] concerning nutrition, physical activity, sleep, stress management, social relationships, adherence to treatment, environmental interactions and other determinants of health. These actions are influenced by personal capability, motivation, opportunity, culture, community and socioeconomic conditions [25].

6. Evaluation and feedback

Implementation generates experiences and outcomes that should be systematically evaluated. People contribute essential feedback regarding feasibility, acceptability, barriers, facilitators and outcomes that may not be captured by conventional clinical measures alone [6].

7. Continuous improvement

Knowledge derived from evaluation informs future research [6] [5], clinical practice, policy development and community initiatives. The cycle then recommences with improved understanding, creating a continuously learning ecosystem that progressively narrows the Evidence-to-Health Gap.

This framework positions partnership not as an isolated activity but as the connective mechanism operating throughout every stage of health creation. A comoparison of the existing and proposed framework is listed in Table-1.

#+Table 1: Conventional Perspeective vs. proposed collaborative paradigm

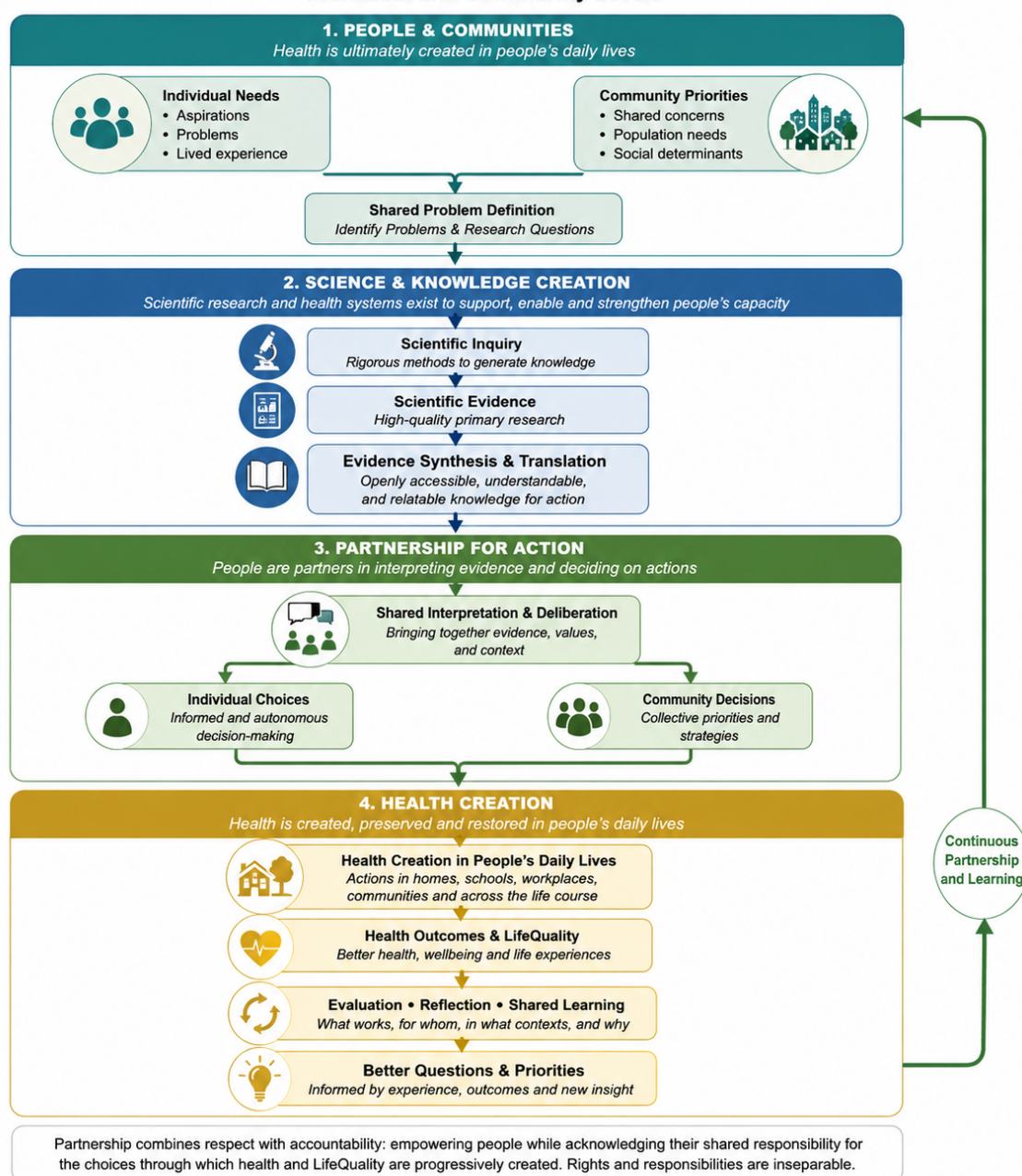
Conventional evidence translation	Partnership model proposed in this Perspective
Research primarily determines priorities.	Priorities are informed jointly by researchers, profession
People participate mainly during implementation.	People participate throughout the evidence-to-action co
Evidence is disseminated to intended users.	Evidence is co-translated into accessible, understandable
Clinical encounters are the principal site of implementation.	Daily life is recognised as the principal site of health cre
Outcomes are measured primarily through clinical indicators.	Outcomes include health, behaviour, participation and l
Learning occurs mainly within research and healthcare institutions.	Learning occurs continuously across individuals, commu
Translation is often viewed as a linear process.	Translation is viewed as a continuous, adaptive learning

Note: The comparison synthesises concepts derived from evidence-based medicine, implementation science, knowledge translation, learning health systems and shared decision making while highlighting the distinctive contribution of the present Perspective [4] [5] [13] [6] [21].

A flow diagram of how individuals and communnities will participate in evidence building and its utilization for health is at Figure-1.

Figure 1. Partnership Across the Evidence-to-Action Continuum at levels of individuals and com-

Figure 1. Partnership Across the Evidence-to-Action Continuum at Individual and Community Levels



Note: The

munity

figure presents the conceptual framework proposed in this Perspective. It illustrates how people and communities participate throughout the evidence-to-action continuum, from identifying priorities and defining research questions to generating evidence, translating knowledge, shared deliberation, health creation in daily life, and continuous learning. Existing disciplines such as evidence-based medicine, implementation science, shared decision making and learning health systems contribute to this continuum, while partnership with people provides the integrating principle.

#+Definitions

For clarity and consistency, the following definitions are used throughout this Perspective.

Evidence-to-Health Gap

The difference between what scientific evidence demonstrates can improve health and the health outcomes ultimately realised by individuals and populations through everyday life.

Health Creation

The continuous process through which health is created, preserved and restored through interactions among individuals, communities, healthcare professionals, health systems and the broader determinants of health, with its

ultimate expression occurring in people’s daily lives.

Partnership

A collaborative relationship in which people, researchers, healthcare professionals, health systems and communities contribute their respective knowledge, expertise and responsibilities throughout the evidence-to-action continuum to improve health and LifeQuality. Partnership operates across every stage of the continuum rather than at a single point within it. Scientific research, healthcare professionals, health systems, communities and people remain continuously connected through reciprocal learning and shared responsibility.

People

Individuals, families, carers and communities who actively participate in creating, preserving and restoring health.

LifeQuality

The overall quality of life experienced by individuals as they pursue meaningful, productive and fulfilling lives. The term encompasses, but is not limited to, health-related quality of life and recognises physical, mental, social and functional well-being as interconnected dimensions of human flourishing.

Evidence-to-Action Continuum

The continuous process through which health priorities are identified, evidence is generated, synthesised, interpreted, implemented, evaluated and refined to create better health and improved LifeQuality.

5 Historical Evolution and Existing Foundations

The partnership model proposed in this Perspective does not emerge in isolation. Rather than introducing an entirely new paradigm, it seeks to integrate and extend several influential movements that have progressively reshaped thinking about health, healthcare and the translation of scientific knowledge into practice. Over the past five decades, disciplines including evidence-based medicine, health promotion, public health, implementation science, shared decision making, lifestyle medicine, participatory research [15] and learning health systems have each contributed important insights into how health can be improved [4] [28] [5] [21] [6] [10]. Collectively, these developments reveal an evolution from viewing people primarily as recipients of healthcare towards recognising their increasingly active role in health creation [19].

Although these disciplines have developed largely independently and with different objectives, they converge around a common recognition: lasting improvements in health require more than scientific discovery and effective healthcare delivery. They require meaningful engagement [19] with the people whose lives ultimately determine whether evidence is translated into better health. The partnership framework proposed here builds upon these foundations by integrating them around a single organising principle focussed on the people as primary agents of health creation.

5.1 Recent Historical Evolution

During much of the twentieth century, healthcare was predominantly organised around the diagnosis and treatment of disease. Scientific progress produced extraordinary advances in microbiology, pharmacology, surgery and diagnostic technologies, contributing to substantial reductions in mortality and improvements in life expectancy [2]. Healthcare systems evolved accordingly, focusing primarily on professional expertise, hospital-based care and the management of acute illness [1].

As patterns of disease shifted towards chronic, multifactorial conditions, it became increasingly apparent that medical care alone could not fully explain differences in health outcomes. The publication of Lalonde’s **A New Perspective on the Health of Canadians** [33] in 1974 marked an important conceptual turning point by proposing that health is influenced not only by healthcare but also by human biology, environment and lifestyle. This broader perspective was subsequently reinforced by the World Health Organization’s Ottawa Charter [28] for Health Promotion, which emphasised enabling people to increase control over and improve their health.

Subsequent decades witnessed growing recognition of the social determinants of health, behavioural science, community participation and health promotion as essential complements to clinical care. [23] [24] [27] Simultaneously, advances in evidence-based medicine [4] [30] strengthened the scientific basis of healthcare, while implementation science [5] [26] [13] emerged to address the persistent challenge of translating evidence into routine practice. More recently, learning health systems [6], patient and public involvement in research [17] [32], citizen science and participatory approaches have further expanded opportunities for collaboration between science and society. (11-15)

Despite these important advances, many developments have evolved within disciplinary boundaries, each addressing specific components of the broader evidence-to-action continuum. The partnership model proposed here

seeks to provide an overarching framework that connects these complementary traditions while preserving their individual strengths.

5.2 Evidence-Based Medicine

Evidence-based medicine (EBM) transformed healthcare by promoting the conscientious, explicit and judicious use of the best available evidence in clinical decision making [4]. Through systematic reviews [30], critical appraisal and evidence-based guidelines, EBM substantially improved the reliability, transparency and consistency of healthcare decisions. Importantly, contemporary formulations of evidence-based medicine recognise that high-quality care requires the integration of three complementary elements: the best available research evidence, clinical expertise and patient values [30] and preferences. This evolution has already moved EBM beyond a purely evidence-centric model towards one that acknowledges the importance of partnership within clinical encounters.

The present Perspective fully embraces the principles of evidence-based medicine. However, it proposes that the endpoint of evidence-based practice should not be viewed as evidence-informed clinical decisions alone, but as improvements in health and LifeQuality realised through people’s daily lives. Partnership therefore extends the principles of EBM beyond clinical encounters to encompass the entire evidence-to-action continuum.

5.3 Lifestyle Medicine

Lifestyle Medicine has demonstrated that many of the leading causes of premature mortality and chronic disease are substantially influenced by modifiable behaviours [10] [11] including nutrition, physical activity, sleep, stress management, avoidance of tobacco, moderation of alcohol consumption and positive social relationships. The evidence [34] [2] supporting these interventions is now extensive and continues to expand.

Yet Lifestyle Medicine also illustrates the central challenge addressed in this Perspective. Although effective lifestyle interventions are increasingly well established, sustained behaviour change [25] [27] remains difficult for many individuals and populations. The existence of compelling evidence alone rarely guarantees long-term implementation.

This observation reinforces our key propositions. Lifestyle Medicine therefore provides one of the strongest practical examples of why partnership is essential. Scientific evidence identifies effective behaviours, healthcare professionals provide guidance and support, health systems create enabling services, but individuals ultimately integrate these recommendations into everyday living within the context of their own values, opportunities and constraints.

Rather than viewing adherence primarily as patient compliance, the partnership framework emphasises collaborative health creation in which people actively participate in understanding, adapting and sustaining evidence-informed behaviours that improve health and LifeQuality.

5.4 Implementation Science

Implementation science emerged in response to the well-recognised delay between scientific discovery and routine clinical practice. [5] [26] [13] The discipline has generated valuable frameworks for understanding barriers and facilitators to evidence uptake, identifying implementation strategies and evaluating their effectiveness [35] across healthcare settings.

The present Perspective is closely aligned with implementation science but differs in the scope of the outcome under consideration. While implementation science has traditionally focused on integrating evidence within healthcare organisations, professional practice and service delivery; in contrast, the Evidence-to-Health Gap extends beyond implementation within healthcare systems to encompass whether evidence ultimately contributes to healthier lives.

This distinction does not diminish the importance of implementation science; rather, it broadens its endpoint. Successful implementation should ultimately be judged not only by adoption within healthcare organisations but also by the degree to which evidence supports people in creating, preserving and restoring health in everyday life.

5.5 Shared Decision Making

Shared decision making represents one of the clearest expressions of partnership within contemporary clinical practice [21] [36]. By integrating scientific evidence with professional expertise and patient values, it promotes informed decisions that respect autonomy while improving communication, trust and satisfaction.

The partnership model proposed here builds directly upon this foundation while extending its scope beyond individual clinical decisions. People participate not only in choosing among treatment options but also in identifying

research priorities, generating evidence, interpreting findings, translating knowledge, implementing evidence and continuously improving health-creating actions.

Shared decision making therefore represents an essential component of partnership but not its full expression. Partnership begins before clinical encounters and continues long after they conclude.

5.6 Learning Health Systems

Learning health systems seek to create continuous cycles in which clinical care, data generation, research and quality improvement mutually reinforce one another [6]. Digital technologies, electronic health records and real-world evidence [37] increasingly enable healthcare systems to learn from routine practice and improve continuously.

The partnership framework complements this vision by recognising people not merely as sources of data but as active participants in learning. Their lived experiences, preferences, contextual knowledge and feedback contribute valuable forms of evidence that enrich scientific understanding and improve implementation.

Consequently, learning extends beyond healthcare institutions to include communities and everyday life, creating a broader ecosystem of continuous improvement centred on health creation rather than healthcare delivery alone.

5.7 Public Health and Health Promotion

Public health has long recognised that health is shaped by environmental, social, economic and behavioural determinants extending well beyond healthcare services [28] [23]. Health promotion similarly emphasises enabling people and communities to increase control over the determinants of health through supportive environments [28], healthy public policies and community action.

The partnership model proposed in this Perspective is strongly aligned with these principles. However, it seeks to strengthen the explicit connection between health promotion and evidence translation. Scientific evidence achieves its greatest value when it becomes understandable, relatable and actionable [24] for the people whose lives it is intended to improve.

This emphasis also recognises that enabling healthier choices requires supportive environments and equitable opportunities. Partnership therefore does not imply that individuals bear sole responsibility for health. Rather, it acknowledges shared responsibility among governments, communities, healthcare systems, professionals and people themselves.

People centered health research and practice model is in line with the mission statement of Public Health [38], which acknowledges the role of the discipline is to create conditions [38] in which people and communities can fulfill their mission to be healthy.

5.8 Rights-Based Approaches

Contemporary healthcare increasingly recognises health as a fundamental human right and emphasises respect for dignity, autonomy, participation and equity [39] [40] [41]. Rights-based approaches have contributed substantially to person-centred care, patient engagement and inclusive health policy.

The present Perspective fully supports these principles while proposing that rights and responsibilities are inseparable. Respect for individual autonomy is accompanied by recognition that people share responsibility for the choices through which health and LifeQuality are progressively created. At the same time, institutions retain corresponding responsibilities to provide trustworthy evidence, equitable healthcare, supportive environments and opportunities that enable healthier lives.

Partnership therefore combines respect with accountability: empowering people while acknowledging their shared responsibility for the choices through which health and LifeQuality are progressively created. This reciprocal understanding avoids both paternalism and excessive individualism, recognising health creation as a shared societal endeavour.

5.9 Integrating Existing Foundations

Taken individually, each of these disciplines addresses an important component of the evidence-to-action continuum. Evidence-based medicine strengthens scientific validity. Lifestyle Medicine demonstrates the importance of everyday behaviours. Implementation science improves translation into practice. Shared decision making enhances clinical partnership. Learning health systems enable continuous improvement. Public health and health promotion address the broader determinants of health. Rights-based approaches reinforce dignity, participation and equity.

The partnership model proposed in this Perspective does not seek to replace these established fields. Rather, it provides an integrating conceptual framework that connects them around a shared objective: enabling people to create, preserve and restore health throughout their daily lives.

Viewed collectively, these disciplines suggest an ongoing evolution in healthcare thinking from treating disease, to delivering evidence-based care, to implementing evidence effectively, and ultimately towards partnering with people in health creation. The proposed framework positions partnership as the unifying principle that links scientific discovery, professional expertise, supportive systems and people's everyday actions into a continuously learning ecosystem capable of narrowing the Evidence-to-Health Gap and improving LifeQuality.

6 Partnership in Practice: Role of Partners in Health

If the Evidence-to-Health Gap is partly explained by insufficient partnership with people throughout the evidence-to-action continuum, then partnership should become a practical organising principle rather than simply an ethical aspiration. Its implications extend across research, healthcare delivery, public health, education, digital health and health policy.

Within research, partnership begins by involving people in identifying priorities, refining research questions and selecting outcomes that reflect what matters most in everyday life. Patient and public involvement, citizen science and community-based participatory research have demonstrated [16] [15] [32] [17] [19] [18] that meaningful engagement can improve the relevance, acceptability and societal value of research without compromising scientific rigour. The present Perspective extends these approaches by proposing that partnership should be considered throughout the complete evidence-to-action continuum rather than at isolated stages of the research process.

Partnership for health framework seeks to strengthen existing institutions, disciplines or professional expertise by creating better partnerships between science and society. The goal is to increase people's capacity to create, preserve and restore health through continuous partnership between science and society. Science serves society most effectively when society participates in the creation, translation and responsible application of scientific knowledge. This Perspective therefore invites discussion not only about how evidence is generated, but also about how relationships between science and society might evolve to enable evidence to become meaningful action.

For healthcare professionals, partnership involves moving beyond the traditional model of information delivery towards collaborative health creation. Clinical expertise remains indispensable, but its impact is strengthened when recommendations are developed through dialogue, adapted to personal circumstances and supported by shared decision making [21] [36], health coaching [42] and longitudinal relationships that recognise the realities of daily life. In this model, clinicians become not only providers of care but also facilitators of people's capacity to create, preserve and restore health.

Health systems similarly play a critical enabling role. Their responsibility extends beyond providing high-quality clinical services to creating environments that support prevention, continuity of care [1] [6], interdisciplinary collaboration and equitable access to trustworthy information. Digital technologies, electronic health records, telehealth and artificial intelligence [43] offer unprecedented opportunities to personalise evidence, facilitate communication and support continuous learning. However, technology should strengthen rather than replace human partnership. Digital innovation achieves its greatest value when it empowers people to understand, apply and contribute to evidence-informed health creation.

Public health agencies and governments also occupy essential positions within the partnership framework. Many determinants of health including education, employment, housing, transportation, food systems and environmental conditions lie beyond the direct influence of healthcare services. Policies that create healthier environments enable individuals to make healthier choices [28] [23] [38] and reduce structural barriers that contribute to health inequities. Partnership therefore requires coordinated action across sectors, recognising that health creation is a shared societal endeavour.

Communities provide another indispensable dimension of partnership. Families, schools, workplaces, voluntary organisations and local networks influence health behaviours through social support, shared norms and collective action. Community engagement [32] enhances trust, facilitates dissemination of evidence and strengthens resilience, particularly among populations facing social or economic disadvantage. These social assets should be recognised as integral components of the evidence-to-action continuum rather than external influences upon it. For, ultimately, individuals integrate scientific evidence, professional guidance, social support and personal experience into countless decisions concerning nutrition, physical activity, sleep, stress management, medication adherence, social relationships and other determinants of health.

Finally, the challenge is not merely to generate better evidence, but to build better partnerships through which scientific knowledge is translated into informed action, healthier lives and improved LifeQuality.

7 Operationalising Partnership: LEAP framework

To operationalise the partnership principles advanced in this Perspective, we propose the Learning and Evidence for Action and Practice (LEAP) framework. LEAP is conceived as a framework to build open partnership centered around people and researchers for identifying research priorities, generating and interpreting evidence and its informed use and dissemination for progressively improving LifeQuality.

LEAP is proposed as an open partnership for translating meaningful questions into rigorous evidence, evidence into understanding, understanding into informed action, and action into better LifeQuality through continuous shared learning. Unique, distinguishing features of LEAP are: Partnership begins before research questions. It addresses LifeQuality instead of Health alone. Democratizing participation, not science. Evidence Translation framework. Rights and responsibilities. Existing explanations remain valid but incomplete. It is built on Trust. People co-creators of knowledge, not merely users of knowledge. Trust develops when people are respected not only as recipients of evidence but also as partners in defining problems, interpreting knowledge and creating solutions. Follows a continuous learning cycle. Builds on historical evolution. It extends open invitation for scientific refinement.

7.1 Placing health options as a Menu before the people

Evidence for health is often standardized, with little choice to the people to choose from a menu as per their circumstances or inclination. These are presented as guidelines, rather than choices. To build a true partnership centered on people, options for health decisions such as lifestyle choices need to be presented with evidence, and let people take the call. To address this gap, we propose a LifeQuality Evidence-Based Options Platform (LEBOP) is proposed as one of the citizens' face of LEAP that presents transparent options for health building choices based on varied life conditions, preferences and values. Such an approach has the potential to preserve users' autonomy, support partnership with people and thereby strengthen the translation of evidence into sustained improvements in health and well-being through healthy behaviours.

8 Proposed LEAP Research Agenda

The partnership hypothesis advanced in this Perspective is intentionally conceptual and therefore requires empirical evaluation. LEAP offers several priorities for future research.

- Research Agenda
- Research questions.
- Pilot studies.
- Living evidence.
- Collaborative evidence synthesis.
- N-of-1 learning.
- Citizen participation.
- AI-assisted translation.
- International collaboration.

Initially, the following research questions can be considered:

First, the concept of the **Evidence-to-Health Gap** should be operationalised and measured. Existing implementation outcomes [44] [26] evaluate adoption, fidelity and sustainability within healthcare settings, but comparatively few metrics assess whether evidence ultimately translates into improved health and LifeQuality in everyday living.^{11,12^} Developing valid measures of this broader outcome would enable systematic evaluation of partnership-based approaches.

Second, future studies should examine whether partnership throughout the evidence-to-action continuum improves outcomes beyond conventional implementation strategies [5] [13]. Comparative evaluations could investigate whether involving people in priority setting, evidence generation, knowledge translation and continuous learning improves the relevance, acceptability, uptake and sustainability of evidence-informed interventions.

Third, methods are needed to synthesise evidence into forms that are openly accessible, understandable and relatable to people’s situations and needs without compromising scientific integrity. Advances in digital communication, artificial intelligence [43] [24] and personalised decision support provide new opportunities for translating increasingly complex evidence into practical knowledge that supports informed action.

Fourth, research should explore methods for integrating experiential knowledge [17] [32] with scientific evidence. People’s lived experiences provide contextual insights that complement quantitative research and may identify barriers, facilitators and outcomes not readily captured by traditional study designs. Appropriate methodological frameworks are needed to ensure that experiential knowledge enriches rather than replaces scientific evidence.

Fifth, longitudinal studies should evaluate whether partnership contributes to sustained improvements in health behaviours, clinical outcomes, health equity [17] [32] and LifeQuality. Such evaluations should include diverse populations and health systems to determine the generalisability and scalability of the proposed framework.

Finally, conceptual work should further refine the relationship between partnership, implementation science [5] [6] [28] [10], learning health systems, public health and Lifestyle Medicine. Rather than creating another isolated discipline, future scholarship should seek greater theoretical integration across these complementary fields.

Possible results framework from the adoption of a people centered health partnership span a series of activities from research to informed choices to better health.

#Table-2: Results expected from people centered health framework

Level	LEAP
Outputs	Meaningful research priorities, rigorous evidence, evidence syntheses, accessible knowledge, decision-support too
Outcomes	Improved understanding, greater agency, informed choices, stronger capability, opportunity and motivation, sust
Impact	Better health, improved LifeQuality, reduced Evidence-to-Health Gap, continuously learning communities and h

9 Discussion

This Perspective proposes that the persistent **Evidence-to-Health Gap** may be partly explained by insufficient partnership with people throughout the evidence-to-action continuum. Since people are both the agents and objects of health creation and preservation, and science exists to solve societal problems, a framework of partnership between the two is likely to be empowering, more efficient, participative and beter compliant. Expectandly, health and qiality of lifes would gain.

The proposed framework is founded upon the central role of people as the central agents in the creation of their health. From this perspective, scientific research, healthcare professionals and health systems exist to support, enable and strengthen people’s capacity to create, preserve and restore health.

The proposed framework is intended to empower people by strengthening their capacity to participate meaningfully in generating, interpreting and applying evidence for their own health and well-being. At the same time, partnership combines empowerment with accountability by recognising that individuals share responsibility for the choices through which health and LifeQuality are progressively created, while acknowledging that these choices are shaped by social, cultural and economic circumstances.

The proposed framework also broadens the endpoint of evidence translation. Conventional models frequently focus on whether evidence is generated, disseminated, adopted or implemented. While these processes remain essential, they represent intermediate rather than ultimate outcomes. The objective of health research is not simply the production of evidence, nor the implementation of guidelines, but measurable improvements in health and **LifeQuality**. From this perspective, health is understood not as an end in itself but as a foundational contributor to LifeQuality, enabling individuals to pursue meaningful, productive and fulfilling lives. This distinction encourages researchers, healthcare professionals and policymakers to evaluate success according to the extent to which scientific knowledge strengthens people’s capacity to create, preserve and restore health.

The framework presented here builds upon rather than challenges established scientific traditions. Evidence-based medicine [4], implementation science [5] [26], shared decision making [21], Lifestyle Medicine [10], public health, health promotion [28], participatory research and learning health systems [6] each contribute indispensable insights into different components of the evidence-to-action continuum. The novelty of the present Perspective lies not in replacing these disciplines but in integrating them around a common organising principle centered around people as principal agents for generating health.

Several limitations should be acknowledged. First, the central partnership hypothesis remains a conceptual proposition that requires empirical validation. Second, the framework intentionally synthesises multiple disciplines and therefore cannot address every theoretical nuance within each field. Third, implementation of partnership

will necessarily differ across healthcare systems, cultures and socioeconomic contexts. Finally, partnership alone cannot eliminate structural inequities or compensate for inadequate resources. It should therefore be viewed as one essential component of a broader systems approach to improving health.

These limitations notwithstanding, the framework offers a coherent conceptual foundation for future research and practice. By connecting existing disciplines through a common understanding of health creation, it provides a basis for developing more integrated approaches to evidence translation that place people at the centre of the health ecosystem.

10 Conclusion

The proposed partnership with people in health places people at the centre of health creation, preservation and restoration. It is expected to advance all the three determinants of behaviour in the COM-B model – Capability, Opportunity and Motivation [25].

We argue that science should not merely produce knowledge for people; it should produce knowledge with people so that people can transform knowledge into healthier and more fulfilling lives. A people-centred approach to scientific knowledge generation and healthcare practice is expected to improve the translation of evidence into sustained improvements in individual and population health. Science serves society most effectively when society participates in the creation, translation and responsible application of scientific knowledge.

Rather than replacing established disciplines, the proposed framework integrates evidence-based medicine, Lifestyle Medicine, implementation science, public health, health promotion, shared decision making, participatory research and learning health systems within a common conceptual model centred on partnership [4] [5] [28] [21] [6]. In doing so, it reframes the objective of evidence translation from implementing evidence within healthcare systems to enabling people to create healthier, more fulfilling lives.

LEAP is not primarily a behaviour-change framework. It is a partnership framework that enables people and society to co-create, translate and continuously apply scientific knowledge, thereby strengthening people’s capacity to create, preserve and restore health and ultimately improve LifeQuality.

If this perspective stimulates further empirical investigation, conceptual refinement and practical innovation, it may contribute to a broader evolution in healthcare thinking from viewing people primarily as recipients of healthcare towards recognising them as partners in health creation. Narrowing the Evidence-to-Health Gap will require not only better science and better healthcare, but stronger partnerships through which scientific knowledge becomes healthier lives and improved LifeQuality.

Recognising people as partners rather than recipients therefore represents not only a conceptual shift, but also an opportunity to narrow the Evidence-to-Health Gap and improve LifeQuality through stronger partnerships between science and society.

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